PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf**) and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	Effectiveness and safety of weekly therapy versus three-weekly therapy of paclitaxel plus carboplatin in women with ovarian cancer: a protocol of systematic review and meta-analysis
AUTHORS	Qiu, Weitao; Fu, Yu; Dang, Yun; Cai, Bingxin; Tuo, Shumei; Mao, Baohong; Lin, Ru; Liu, Qing; Li, Yilin

VERSION 1 – REVIEW

REVIEWER	Tate, Anne School of Informatics University of Sussex, Primary Care
REVIEW RETURNED	12-Aug-2021

GENERAL COMMENTS	This seems a very useful study and is well described. Just one comment. I assume you mean random *effects* model in the abstract?

REVIEWER	Burbos, Nikolaos Norfolk and Norwich University Hospital
REVIEW RETURNED	11-Oct-2021

GENERAL COMMENTS	Thank you for submitting this protocol for systematic review and meta-analysis. As you acknowleldge a Cochrane systematic review on this topic was published in 2015. However, due to publication of the results of ICON8 trial, there is need for a new systematic review. I have the following comments for the authors: 1. Although subgroup analsis is planned, the authors should consider the effects of chemotherapy regimens in relation to the outcome of surgical cytoreduction. This will be useful to guide clinical practice. For example, in patients that have undergone complete cytoreduction, is weekly treatment of three-weekly therapy more effective? 2. Timing of cytoreductive surgery should be considered. Is weekly treatment more effective in patients undergoing primary surgery or interval cytoreduction?

REVIEWER	Helpman, L Juravinski Cancer Center, McMaster University, Gynecologic Oncology
REVIEW RETURNED	17-Oct-2021

GENERAL COMMENTS	First line treatment for ovarian cancers has not evolved significantly since the 1990s and platinum/taxane combinations remain the drugs of choice for this disease. Several studies have compared different schedules of administration and other reviews

	and meta-analyses have been performed addressing these. While not novel, the analysis planned in this manuscript may certainly present an updated snapshot of current evidence on this topic. Generally speaking, the research protocol is fairly detailed and thought out. I would suggest broadening the search strategy somewhat to include alternate terms for ovarian cancer and for drugs and treatment schedules. I would also suggest a clearer definition of the studies considered for inclusion (RCTs with how many arms? which schedules? what is the minimal patient number? etc). The statistical plan is robust. I appreciate the authors' attention to heterogeneity, risk of bias and their plan to perform sensitivity analyses and to adjust for ethnicity, which has been a potential confounder in the evidence generated on this topic.

VERSION 1 – AUTHOR RESPONSE

Reviewer 1:
This seems a very useful study and is well described. Just one comment. I assume you mean random *effects* model in the abstract?
Response: Thank you for your reminder. We have revised them in the revised manuscript.

Reviewer 2: Thank you for submitting this protocol for systematic review and meta-analysis. As you acknowleldge a Cochrane systematic review on this topic was published in 2015. However, due to publication of the results of ICON8 trial, there is need for a new systematic review. I have the following comments for the authors:
1. Although subgroup analsis is planned, the authors should consider the effects of chemotherapy regimens in relation to the outcome of surgical cytoreduction. This will be useful to guide clinical practice. For example, in patients that have undergone complete cytoreduction, is weekly treatment of three-weekly therapy more effective?
2. Timing of cytoreductive surgery should be considered. Is weekly treatment more effective in patients undergoing primary surgery or interval cytoreduction?
Response: Thank you and we agreed with your comments, and we further added subgroup analyses and revised as follows:
“We will perform subgroup analyses for dose-dense (increased cumulative dosage) versus metronomic (similar cumulative dosage) dosing schedule, chemotherapy regimens (cytoreductive (or debulking) surgery to remove malignant tissue versus no cytoreductive surgery), timing of cytoreductive surgery (primary surgery or interval cytoreduction), studies conducted in Asia or with a majority of Asian patients versus studies conducted in Western countries and the survival rate at different mean follow-up duration.”

Reviewer 3: First line treatment for ovarian cancers has not evolved significantly since the 1990s and platinum/taxane combinations remain the drugs of choice for this disease. Several studies have compared different schedules of administration and other reviews and meta-analyses have been performed addressing these. While not novel, the analysis planned in this manuscript may certainly present an updated snapshot of current evidence on this topic.
Generally speaking, the research protocol is fairly detailed and thought out. I would suggest broadening the search strategy somewhat to include alternate terms for ovarian cancer and for drugs and treatment schedules. I would also suggest a clearer definition of the studies considered for inclusion (RCTs with how many arms? which schedules? what is the minimal patient number? etc).
The statistical plan is robust. I appreciate the authors' attention to heterogeneity, risk of bias and their plan to perform sensitivity analyses and to adjust for ethnicity, which has been a potential confounder in the evidence generated on this topic.

Response: Thank you and we agreed with your comments. We further improved our search strategy and revised the details of searching strategy in each database in the revised online supplemental file 2 as follows:
5 (paclitaxel or taxol or praxel or anzatax or nsc125973 or nsc 125973 or paxene or onxol or abraxane or "abi 007" or asotax or bristaxol or xytotax).mp.

And we revised the search terms for retrieval of studies part of manuscript as follows:
“Search terms will combine MeSH and full text terms related to “Paclitaxel”, “taxol”, “anzatax”, “paxene”, “onxol”, “abraxane”, “praxel”, “Ovarian Neoplasms”, “ovarian cancer”, “ovarian adenocarcinoma”, “Drug Administration Schedule”, “dose-dense” and “weekly”.”

Thanks for your suggestion for a clearer definition of the studies considered for inclusion. And we will include all types of RCTs without limitation the arm of the RCTs. And we will not limit the minimal patient number of studies. We further clearly defined the participants as follows:
“Adult women, 18 years or older, newly diagnosed with ovarian cancer confirmed by pathology, the expected survival time is at least six months, the living condition score (Kamofsky score, KPS) >60, without intracranial and bone metastases, and the function of heart, liver, kidney and bone marrow is normal.”

Many thanks again for all of the constructive suggestions!

VERSION 2 – REVIEW

REVIEWER	Burbos, Nikolaos Norfolk and Norwich University Hospital
REVIEW RETURNED	02-Dec-2021

GENERAL COMMENTS	Thank you for submitting a revised version of the protocol. I have the following comments: 1. Please review the sentence added in page 4 at the Subgroup analysis and investigation of heterogeniety: '...chemotherapy regimens (cytoreductive (or debulking) surgery to remove malignant tissue versus no cytoreductive surgery), timing of cytoreductive surgery(primary surgery or interval cytoreduction'. 2. The subgroup analysis should include assessment of dose dense chemotherapy versus metronomic dosing in patients that underwent complete tumour resection, optimal and suboptimal resection. These results will have a clinical implications following cytoreductive surgery for ovarian cancer.

VERSION 2 – AUTHOR RESPONSE

Reviewer 2:

1. Please review the sentence added in page 4 at the Subgroup analysis and investigation of heterogeniety: '...chemotherapy regimens (cytoreductive (or debulking) surgery to remove malignant tissue versus no cytoreductive surgery), timing of cytoreductive surgery(primary surgery or interval cytoreduction'.

2. The subgroup analysis should include assessment of dose dense chemotherapy versus metronomic dosing in patients that underwent complete tumour resection, optimal and suboptimal resection. These results will have a clinical implications following cytoreductive surgery for ovarian cancer.

Response: Thank you and we agreed with your comments, and we further added subgroup analyses and revised as follows:

“We will perform subgroup analyses for dose-dense (increased cumulative dosage) versus metronomic (similar cumulative dosage) dosing schedule in patients that underwent completely tumour resection, optimally and suboptimally resection debulked surgery, timing of cytoreductive surgery (primary debulking surgery (PDS) or Interval debulking surgery (IDS)), studies conducted in Asia or with a majority of Asian patients versus studies conducted in Western countries, the survival rate at different mean follow-up duration.”

Many thanks again for all of the constructive suggestions!